G2211 Add-on Code: What It Is and When To Use It

The AAFP is closely monitoring implementation of G2211 and will update this page as CMS releases more information.

The care you provide your patients is complex and comprehensive. G2211 will help you get paid more accurately for it. Starting January 1, 2024, use this new add-on code alongside office/outpatient evaluation and management (E/M) codes to receive additional payment for the high-value visits you provide.

- What is G2211?
- When to use G2211
- How to start billing for G2211
- Resources for members
- Additional FAQs

Get the facts about why G2211 is needed.

VIEW FACTS

What is G2211?

Healthcare Common Procedure Coding System (HCPCS) add-on code G2211 reflects the time, intensity, and practice expense resources involved when physicians provide office/outpatient visits that build longitudinal relationships with patients and address the majority of a patient's health care needs with consistency and continuity over longer periods of time.

The Centers for Medicare & Medicaid Services (CMS) created code G2211 to better account for the resource costs associated with visit complexity inherent to primary care and other longitudinal care.

The 2024 national Medicare allowable for G2211 is $16.05.

CMS Code Descriptor: G2211

Visit complexity inherent to evaluation and management associated with medical care services that serve as the continuing focal point for all needed health care services and/or with medical care services that are part of ongoing care related to a patient's single, serious condition or a complex condition. (Add-on code, list separately in addition to office/outpatient evaluation and management visit, new or established)

When to use G2211

HCPCS code G2211 is payable starting January 1, 2024.

G2211 is an add-on code that may be reported with new and established patient office/outpatient evaluation and management (E/M) services.

✓ Use the add-on code when you are the continuing focal point for all health care services the patient needs. Per CMS, the relationship between the patient and the physician is the determining factor of when the add-on code should be billed.

✓ As of 2026, G2211 may also be reported with home or residence E/M services (CPT codes 99341-99350).

Do **not** use G2211 when:

✗ Your relationship with the patient is of a discrete, routine, or time-limited nature. For example, a physician who sees a patient for an acute concern should not report HCPCS G2211 if they have not also assumed responsibility for the patient's ongoing medical care or do not plan to take responsibility for subsequent, ongoing medical care with consistency and continuity over time.

***FPM* covers 2026 Medicare Fee Schedule and CPT Coding Update**

The 2026 Medicare Physician Fee Schedule includes several provisions that should boost primary care. Read the article.

Understand how to properly document and code for E/M services.

E/M CODING FAQS

How to start billing for G2211

Update your EHR and/or billing systems to reflect the 2024 Medicare physician fee schedule.

- Verify G2211 is added as part of the updates. Usually practice management or billing and coding staff can help with this.
- Since deductible and coinsurance apply, inform patients that there may be an additional charge on their bill.

Report HCPCS code G2211 with office visits where you have assumed or intend to assume responsibility for the patient's ongoing medical care.

Educate your administration and coding staff about the importance of G2211.

Let them know that:

- Studies show that primary care office visits are more complex than those conducted by other specialties. Primary care physicians pack highly complex care into brief visits by managing all a patient's acute and chronic conditions, providing ongoing preventive services, and counseling, and addressing behavioral health challenges and unmet social needs. To fully account for this additional complexity and related practice costs, G2211 is needed.
- Medicare officials agree that updated office visit E/M payments do not fully account for the more complex, whole-person care provided by primary care physicians. Existing coding processes are better at denoting procedures than at describing cognitive services such as continuous, comprehensive primary care.
- Current E/M coding fails to account for many of the unique services and resources that primary care physicians provide or reflect their extraordinary role in coordinating care for their patients. The typical primary care physician caring for Medicare patients must coordinate care with 229 other physicians working in 117 practices.

Member-exclusive G2211 resources

Use these five free member-only resources to advocate for payment of G2211, track private payer coverage, and navigate claims.

NOT A MEMBER? JOIN NOW. >

G2211 Update and Infographic | *FPM* Jan/Feb 2025

Family physician authors developed an article and infographic that make G2211 less confusing to navigate in 2025.

National G2211 Private Payer Coverage Matrix | *Updated June 17, 2024*

This table is updated as the AAFP receives requested coverage details.

Verifying Coverage and Navigating Denials Guide

Use this guide when you want to contact payers about their G2211 policies or need to address an inappropriate claim denial.

Letter Template: Physician to Payer

Use this letter template to craft a message to private payers about the importance of coverage and payment of G2211.

Letter Template: Physician to Employer

Employed physicians can use this letter template to encourage their organizations to reach out to private payers with support for G2211 coverage.

Template: G2211 Private Payer Coverage Matrix

Keep track of your payers' policies on G2211 with a table to help you keep coverage status, resource links, and contact information organized.

G2211 Advocacy Talking Points

Members and AAFP chapter staff are encouraged to partner and advocate for coverage and payment of G2211. Use these talking points to educate stakeholders on the purpose of the add-on code and the complexity of primary care.

Additional FAQs

- What needs to be documented?
- Do private payers pay for G2211?
- What constitutes a "single, serious condition or complex condition"?
- Can federally qualified health centers (FQHCs) and rural health clinics (RHCs) bill for G2211?
- Can G2211 be billed under the primary care exception?
- Is G2211 limited to certain specialties?
- How often can G2211 be billed?
- Do deductible and coinsurance apply?
- Can G2211 be billed with telehealth visits?
- Are there any examples of when G2211 can be used?
- When do I need to use modifier 25?
- What G2211 resources does CMS have?

What's next

Safeguard G2211

Congress must enact long-term Medicare payment reforms. Use the AAFP's Speak Out tool to urge your lawmakers to modernize payment.

How to Use Modifier 25

Learn more about modifier 25 and how it affects G2211.

Getting Paid Blog

Read *FPM*'s Getting Paid blog to keep up on payment schedule changes and get tips to make coding more accurate and efficient.

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